

geha-coverage-policy-rytelo.pdf

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*=G.E.H.A Rytelo (imetelstat).

Original Effective Date: 7/15/2024

Revision Date(s): 7/15/2024; 11/20/2024; 1/1/2025; 12/19/2025

Review Date: 12/19/2025

Policy type: Medical Necessity.

Line of Business: All G.E.H.A Medical Plans.

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability..

Preference	Requires Prior Auth	Drug Name	HCPCS Description Code	
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Preferred	Yes	Reblozyl (luspatercept)J0896		Injection, luspatercept-aamt, 0.25 mg
Non-Preferred	Yes	Rytelo (imetelstat)	0870	Injection, imetelstat, 1 mg
Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents prior to approval of a non-preferred product, when clinically appropriate. Preferred product requirements apply to new starts only.				
Step Therapy applies to all overlapping compendia supported indications/regimens..				

Indication Specific Approval Criteria:

Myelodysplastic Syndrome (MDS)

Member is 18 years of age or older; AND

Member's MDs is classified as IPss low- to intermediate-1 risk; AND

Member's absolute neutrophil count is $1.5 \times 10^9/L$ or greater within the last 30 days; AND.

Member's platelets are $75 \times 10^9/L$ or greater within the last 30 days; AND

Member has no prior treatment with either lenalidomide OR hypomethylating agents; AND.

Member does not have del(5q) cytogenic abnormality; AND

Member is transfusion-dependent requiring 4 or more units of red blood cells in previous 8 weeks;AND

Member has ring sideroblasts (Rs) of 15% or more (or 5% or more with SF3B1 mutation); AND

Member has serum EPO $> 500 \text{ mU/mL}$; OR

Member has serum EPO of 500 or less; AND

Member has lost response to luspatercept; OR.

Member has RS less than 15% (or RS less than 5% with SF3B1 mutation); AND

Member has serum EPO greater than 500 mU/mL ; AND

Member and no response to immunosuppressive therapy (IST), did not tolerate IST, relapsed after IST or is a poor candidate for IST; OR.

Member has serum EPO of 500 mU/mL or less; AND

Member has lost response to erythropoietin-stimulating agents (EsAs); OR

Member has failed to respond to ESA therapy; OR.

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Member is ineligible for ESA therapy; AND

Member has lost response to luspatercept

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

National Comprehensive Cancer Network@ (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; OR

United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; OR.

Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:.

O Clinical Pharmacology (Strong For); OR.

OWolters Kluwer Lexi-Drugs (Level A); OR

Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:.

O American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology; AND

O Meeting abstracts and case reports are excluded from consideration;.

Non-standard protocols may be approved based on unique clinical circumstances;

Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network@ (NCCN), or indication specific peer-reviewed literature;

In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal)..

Billing

Drug Name	HCPCS Code	Description
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Rytelo	J0870	Injection, imetelstat, 1 mg.
Reblozyl	J0896	Injection, luspatercept-aamt, 0.25 mg

G.E.H.A

References

1. Reblozyl [package insert]. Celgene Corporation, a Bristol-Myers Squibb Company, Summit, NJ.Available at: https://www.accessdata.fda.gov/drugsatfda_docs/label/2024/761136s010lbl.pdf 2. Reblozyl. NccN Drugs & Biologics Compendium. Available at: https://www.nccn.org/professionals/drug_compendium/content/

3.Rytelo [package insert]. Geron Corp., Foster City, CA. Available at:https://pi.geron.com/products/Us/pi/rytelo_pi.pdf

4. Rytelo. Nccn Drugs & Biologics Compendium. Available at:

5. Referenced with permission from the NCcN Clinical Practice Guidelines in Oncology (NcCN Guidelines). National Comprehensive Cancer Network, Inc. 2025. All rights reserved. Accessed December 19, 2025. To view the most recent and complete version of the guideline, go online to NCCN.org.

6.Fenau, P., Platzbecker U., et al. Luspatercept in Patients with Lower-Risk Myelodysplastic Syndromes. N Engl J Med 2020;382:140-151; DOI: 10.1056/NEJMoa1908892; VOL. 382 NO. 2.<https://www.nejm.org/doi/full/10.1056/NEJMoa1908892>

7. Platzbecker, U., Santini, V., et al. Imetelstat in patients with lower-risk myelodysplastic syndromes who have relapsed or are refractory to erythropoiesis-stimulating agents (IMerge): a multinational, randomised, double-blind, placebo-controlled, phase 3 trial. The Lancet, Volume 403, Issue 10423,P249-260, January 20, 2024. [https://www.thelancet.com/journals/lancet/article/Plis01406736\(23\)01724-5/abstract](https://www.thelancet.com/journals/lancet/article/Plis01406736(23)01724-5/abstract)

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	Date		Updates	
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	7/10/2024		Policy creation	
	7/15/2024		Policy adoption by P&T	
	11/20/2024		For 1/1/2025 - update to permanent HCPCS Code per CMS HCPCS Update	
	12/15/2025		Policy adopted by G.E.H.A; document updated for universal accessibility	
	12/15/2025		Annual Review	